

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the use of Norethisterone to Delay Menstruation

Summary of NICE CKS Guidelines for Period Delay

Overview

- **Menstrual delay** may be requested for reasons such as holidays, exams, religious events, or special occasions.
- Management depends on whether the woman is using **hormonal contraception** or **not**.

1. Women Not Using Hormonal Contraception

First-line option: Norethisterone

- A synthetic progestogen used to delay menstruation.
- **Dose:** 5 mg **three times daily**, starting **3 days before** expected period.
- Continue for up to **14 days** (or longer under specialist advice).
- Withdrawal bleed usually occurs **2–3 days** after stopping.

Important considerations:

- Not suitable for women with a **history of thromboembolism, liver disease, breast cancer, or undiagnosed vaginal bleeding**.
- Use with **caution** in women with cardiovascular risk factors.
- **Not a contraceptive** — advise additional contraception if needed.

2. Women Using Combined Hormonal Contraceptives (CHC)

Options:

- **Monophasic 21-day CHC pill:**
 - o **Take back-to-back** packs without the usual 7-day break to delay bleeding.
 - o Can be done for up to 3 consecutive packs.

- **Everyday (ED) formulation:**
 - **Skip placebo pills** and continue active pills from next pack.

Note:

- Breakthrough bleeding may occur, especially after extended use.
 - Safe for most users with no additional health risks.
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3. Women Using Progestogen-Only Contraceptives

- **Progestogen-only pills (POP):**
 - **Cannot be used to reliably delay periods.**
 - **Injectables or implants:**
 - Often cause irregular bleeding or amenorrhoea.
 - No role in actively delaying periods.
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Additional Guidance

- **Pregnancy must be excluded** before prescribing for delay.
- For norethisterone, consider interactions (e.g. enzyme inducers may reduce effectiveness).
- **Medroxyprogesterone acetate** may be used **off-label** as an alternative where norethisterone is unsuitable, though less common.

Patient Group Direction

for the supply/administration of

Norethisterone 5mg

to

Delay Periods

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Norethisterone 5 mg is indicated for the short-term treatment to delay menstruation in women aged 18 years and over who wish to postpone their period for personal, social or travel-related reasons.
Inclusion criteria	- Female patients aged 18 years or over. - Regular menstrual cycle. - No contraindications to treatment with progestogens. - Informed consent obtained. - Requesting short-term delay of menstruation (e.g. for travel, events).
Exclusion criteria	- Known or suspected pregnancy. - Breastfeeding. - History of venous thromboembolism (VTE), stroke, or arterial disease. - Liver dysfunction or liver tumour. - Hormone-sensitive cancer (e.g. breast cancer). - Undiagnosed vaginal bleeding or abnormal menstrual cycle. - Male patients - Have hypersensitivity to the active substance or to any of the excipients in norethisterone 5 mg tablets - History of migraine with focal neurological symptoms - Severe pruritus and porphyria - Taking a clinically significant, interacting medicine
Cautions	- Increased risk of VTE; assess individual risk factors. Risk factors include: smoking, long-duration travel, obesity (BMI >30kg/m²), recent surgery. - May affect blood glucose levels in diabetics. - May cause mood changes. - May cause fluid retention. Special care should be taken for patients with conditions which might be aggravated by this factor e.g. epilepsy, migraine, asthma, cardiac dysfunction, renal dysfunction, risk of VTE - Advise on timing of use and that menstruation usually returns within a few days after stopping. - Patients should be advised that norethisterone is not a contraceptive and so another form of contraceptive should be used to avoid pregnancy - Therapy should be discontinued if the following occur: • Jaundice or deterioration in liver function • Significant increase in blood pressure • New onset of migraine-type headache • Pregnancy • Sudden perceptual disorders, e.g. disturbances of vision or hearing • Potential thromboembolic symptom e.g. painful swelling in the leg, sudden pain in the chest, dyspnoea. - Where prolonged immobilisation is likely e.g. following surgery, treatment should not be started until the patient is fully remobilised.
Actions if patient is excluded or declines	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP

treatment	as appropriate
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Details of the medicine

Name, form and strength of medicine	Norethisterone 5mg tablets
Legal category	POM
Storage	Store below 25°C in a dry place away from direct sunlight.
Route/method of administration	Oral administration.
Dose and frequency	One tablet (5 mg) taken three times daily, starting 3 days before the expected onset of menstruation.
Quantity to be administered and/or supplied	Up to 90 tablets
Maximum or minimum treatment period	Usually, up to 17 days but can extend to 28 days if required. Menstruation typically occurs 2-3 days after stopping. A patient should not treat for more than 30 days in total a six-month period.
Adverse effects	Nausea, headache, breast tenderness, mood changes, and breakthrough bleeding. Rare: thromboembolic events, jaundice. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP if appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Inform the patient or carer of the possible side effects and their management Norethisterone is a progestogen hormone. Progestogens are hormones that sustain the lining of the womb (uterus). Normally, at the time before a period, there is a fall in the level of progestogen hormone in the body. When it falls below a certain level, the lining of the womb is shed as a menstrual period. By taking norethisterone tablets (progestogen) the lining of the uterus is sustained until the tablet is stopped. Note: norethisterone taken in this way is not a contraceptive. Menstruation usually follows within 3 days of finishing the treatment. If menstrual bleeding should fail to follow a course of norethisterone, the possibility of pregnancy must be considered.

Key references

● NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2 ● NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources ● NICE CKS Guidance ● British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) NICE ● Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature

Valid from date		Expiry date	
16/2/26		31/10/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		16/2/26
Chris Pilkington	Pharmacist GPhC: 2046322		16/2/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	16/2/26

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